



CLINICAL PERFORMANCE LAB  
The Cheat Sheet Library

VOL III OF IV

# DDx Ranking & Key Findings

How iHuman scores your differential — and  
why most students rank wrong

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The Assessment section is where clinical reasoning shows. Faculty  
scrutinize this part — and the platform's auto-scoring is strict.

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Vol III of IV

# DDx Ranking & Key Findings

How iHuman scores your differential — and why most students rank wrong

## Here's the thing about iHuman's differential diagnosis scoring:

It's not just asking you to list possible diagnoses. It's testing whether you can rank them correctly — most likely first, must-not-miss prominently positioned, and ruled-out conditions explicitly addressed.

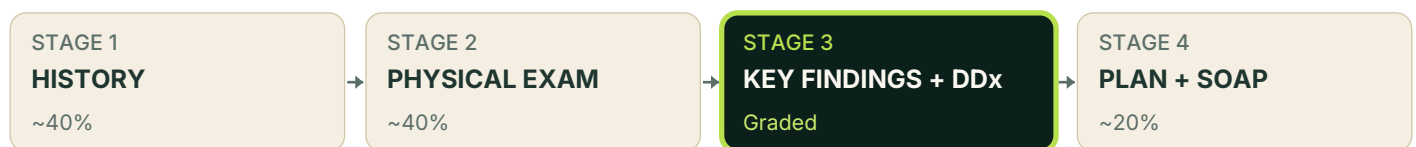
## Get the ranking wrong and the score drops even when you have the right diagnosis on the list.

Faculty notice this. And on top of the auto-scoring, the problem statement is faculty-scored — they're checking whether your 2-3 sentence summary captures the case clinically and uses the right level of medical language.

## This sheet covers how to think about DDx ranking, how to write a strong problem statement, and how to select key findings without padding the list.

These are skills, not memorization. Apply them across every case.

THE IHUMAN CASE FLOW · You are here



## The Three Buckets of DDx

Every differential diagnosis on iHuman falls into one of three categories. Your ranking should reflect this:

**1. Most-Likely Diagnoses** — supported by history + PE + risk factors

Examples: GAS pharyngitis with Centor 4/4; Stage 2 HTN with target organ damage

**2. Must-Not-Miss Diagnoses** — less likely but high consequence if missed

Examples: Meningitis on any headache case; OSA on any HTN/snoring case; appendicitis on RLQ pain

### **3. Ruled-Out (or Less Likely) Diagnoses** — considered and excluded based on evidence

Examples: Hypothyroidism (ruled out by normal TSH); secondary HTN (ruled out by normal renal labs)

**The structure of your DDx list should mirror these buckets** — most likely first, must-not-miss prominently, ruled-out near the bottom with explicit ruling-out rationale.

## **How iHuman Auto-Scores DDx**

**Selection matters.** Each DDx you select scores positive if it's a valid consideration, negative if it's harmful or wildly off-track.

**Ranking matters.** The platform weights how you order the list. The final/primary diagnosis should be ranked #1.

**Must-not-miss matters separately.** A case with a must-not-miss diagnosis (e.g., cluster headache on a migraine case, OSA on an HTN case) scores additional points only if the must-not-miss is explicitly included on your list.

**Verbatim names matter.** iHuman uses a fixed dictionary of diagnosis names. If your platform expects "Acute viral gastroenteritis" and you select "Stomach bug," you get zero. Always pick the exact platform diagnosis name.

# Writing the Problem Statement

## PROBLEM STATEMENT · 3-sentence structure

Faculty-scored. Clinical language only. Brevity wins.

### Who + CC + Duration

**S1** H.H. is a 57-year-old Hispanic male who presents with elevated blood pressure noted at a community health fair, with no previous follow-up.

### Key positives + Risk factors

**S2** Examination reveals BP 172/94 (L) and 178/98 (R), AV nicking on fundoscopy, and laterally displaced PMI; risk factors include 35-pack-year smoking, family history of premature CVD, and high-sodium diet.

### Diagnostic impression + Must-not-miss

**S3** Findings are consistent with Stage 2 essential hypertension with target organ damage; OSA is a must-not-miss given snoring, obesity, and HTN.

## What Faculty Look For

**Clinical language** — no lay terms, no patient quotes

**Inclusion of both positives and risk factors** — not just symptoms

**Naming both the primary AND must-not-miss diagnoses**

**Appropriate medical brevity** — 3 sentences, not 6

## Centor Criteria — Sore Throat Cases

### CENTOR CRITERIA · GAS pharyngitis probability

One point per criterion present (plus age modifier)

|                                                                                    |         |
|------------------------------------------------------------------------------------|---------|
| <input type="checkbox"/> Tonsillar exudate                                         | 1 point |
| <input type="checkbox"/> Tender anterior cervical lymphadenopathy                  | 1 point |
| <input type="checkbox"/> Fever ( $>38^{\circ}\text{C}$ / $100.4^{\circ}\text{F}$ ) | 1 point |
| <input type="checkbox"/> Absence of cough                                          | 1 point |
| <input type="checkbox"/> Age 3–14 (modified Centor)                                | +1      |
| <input type="checkbox"/> Age 15–44                                                 | 0       |
| <input type="checkbox"/> Age $\geq 45$                                             | -1      |

#### INTERPRETATION

- 0–1** Low risk — no testing, no treatment
- 2–3** Test (rapid strep or culture). Treat if positive.
- 4–5** High risk — empiric antibiotics may be appropriate

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## Selecting Key Findings — The Goldilocks Problem

**iHuman gives you a list of 12–20 findings on most cases.** You're asked to select the ones that are present and clinically relevant.

**Too many** (selecting everything): Lowers score — flags poor clinical judgment.

**Too few** (selecting only obvious ones): Lowers score — misses pertinent positives.

**Goldilocks** (5–10 selections on a typical case): Strongest signal of clinical reasoning.

#### Select if it's:

A direct symptom or sign of the working diagnosis

A risk factor that strengthens the diagnosis

A pertinent negative that rules out a serious alternative

**Don't select if it's:**

- Normal/baseline (unless explicitly a pertinent negative for must-not-miss)
- Tangentially related to another finding already selected
- An incidental finding with no clinical bearing on the case

## Must-Not-Miss by Chief Complaint

**Every case has at least one must-not-miss diagnosis.** Faculty award points for including it in the DDx, even when you correctly identify it as less likely than the primary.

**Patterns by chief complaint:**

**Headache** → meningitis, subarachnoid hemorrhage, brain tumor, temporal arteritis (age >50), cluster headache

**Chest pain** → MI, PE, aortic dissection, pneumothorax, pericarditis

**Abdominal pain** → appendicitis, ectopic pregnancy (women of childbearing age), bowel obstruction, AAA (elderly)

**HTN** → OSA, secondary HTN (renal, endocrine), pheochromocytoma

**Sore throat** → GAS pharyngitis, infectious mononucleosis, peritonsillar abscess, epiglottitis (peds)

**Pediatric vomiting/diarrhea** → severe dehydration, bacterial gastroenteritis vs viral, intussusception, pyloric stenosis (infants)

**Pediatric academic decline** → vision impairment, hearing impairment, ADHD, learning disability, depression

**The rule:** When ranking, your must-not-miss goes on the list. It doesn't have to be #1 — but it must be present.

## Common DDx Traps

**⚠ Selecting too many diagnoses**

Picking everything that's even slightly plausible suggests poor clinical judgment. Be selective.

**⚠ Missing the must-not-miss**

Even if you correctly identify it as less likely, the platform expects it on the list.

**⚠ Wrong final-diagnosis ranking**

Your top-ranked DDx should match your final diagnosis. Mismatches cost points.

**⚠ Using non-platform terminology**

iHuman uses fixed diagnosis names. "Severe high blood pressure" doesn't match "Hypertension, Stage 2."

**⚠ Not addressing ruled-out conditions**

When labs come back normal, explicitly rule out the conditions they ruled out. Silent dismissal loses points.

**⚠ Padding the problem statement**

Three sentences. Concise. Clinical language. Faculty want diagnostic reasoning, not a re-summary of the entire HPI.

## Next steps

**Last in the library: Management Plan + SOAP Note** — the part that's 100% faculty-scored. Tests to order, the harmful-flag trap, the 6-part management plan structure, and the SOAP note template that faculty look for.

READY FOR THE FULL GUIDE?

## Walk into your case with the answer key.

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